

Response to the White House Artificial Intelligence Action Plan

AI Action Plan
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This letter is written by AI leaders at UC San Diego Health and UCSF Health and includes co-signatories from other institutions.

On January 23, 2025, President Trump signed Executive Order 14179, "Removing Barriers to American Leadership in Artificial Intelligence," to establish U.S. policy for sustaining and enhancing America's position as a leader in AI through the development of an AI Action Plan focused on human flourishing, economic competitiveness, and national security. At the invitation of the Office of Science and Technology Request for Information, we are writing to share policy ideas for the AI Action Plan with a focus on improving American health and healthcare.

As health AI leaders within two large health systems with nationally recognized AI expertise,¹ we appreciate the renewed attention to considering how AI tools should be regulated, governed, and used in the care of patients. The current regulatory environment is fragmented, with multiple federal entities involved in certain aspects of AI regulation to varying degrees. This has stifled innovation by creating uncertainty among health systems, AI companies, and AI researchers in how to safely evaluate and implement new and groundbreaking AI tools. The absence of clear federal guidance is leading to a patchwork of state-based regulation, which will further slow adoption of health AI. In this letter, we highlight 3 opportunities to improve AI policy as it pertains to healthcare: 1) make AI procurement easier for health systems through improved transparency; 2) streamline software-as-medical device (SaMD) regulation for health AI tools; and 3) lift restrictions on the use of AI-powered translation tools.

1. Make AI procurement easier for health systems through transparency.

We recommend the adoption of basic transparency requirements for AI companies to ensure health systems can make informed purchasing decisions based on technological merit rather than marketing claims.

Currently, the Office of National Coordinator's HTI-1 Final Rule requires AI developed by certified electronic health record (EHR) vendors to disclose 31 "source attributes" about predictive AI systems used to support clinical decision-making.² These source attributes consist of basic facts about a model, like the ingredients of a recipe. We have found the source attributes helpful in screening and evaluating models. Certain information about how an AI model was trained may make it more or less useful to a health system.

We would like to see these same transparency expectations extended to health AI companies, and not just EHR vendors. In the absence of this requirement, AI companies are free to decide which ingredients to share. As a result, health systems often end up making decisions based on misleading marketing claims that are actually artifacts of the way AI tools were trained or tested.

Why this is important: As high-quality evidence begins to link the use of predictive and generative AI with better patient and clinician outcomes, health systems are eager to adopt them. For instance, the deployment of one sepsis predictive AI model at UC San Diego Health reduced death from sepsis by 17%, leading to about 50 lives saved in our health system annually.³ However, this is a best-case scenario. **Health systems often waste a tremendous amount of time and resources testing models internally, only to find that they don't perform adequately or even pose a risk to patients.** One such example involved a sepsis model deployed at multiple health systems⁴⁻⁶ where the model's poor performance was attributed to its reliance on the prescription of intravenous (IV) antibiotics as a predictor of sepsis. This is not helpful clinically because, as you might imagine, once antibiotics are prescribed the physician is already concerned about infection. With dozens of sepsis models currently on the market, health systems have no way to compare basic facts about AI tools (such as which predictors they rely upon), which can be the difference between a model that saves lives and one that wastes valuable clinician time.

2. Streamline software-as-medical-device (SaMD) regulation for health AI tools.

We recommend that a unified approach be developed for overseeing health AI tools, regardless of whether those tools are developed by electronic health record (EHR) vendors, AI companies, or AI researchers. While EHR vendors can develop and deploy AI tools in health systems with minimal oversight, AI companies including startups must navigate the Food and Drug Administration's SaMD processes, which treat many AI tools used for clinical decision support as medical devices.⁷ While well-intentioned, a series of nonbinding guidance documents from the FDA have led to further confusion about which AI tools are considered medical devices and the resulting implications. Health systems are hesitant to adopt technology that is not FDA approved when it is unclear whether it constitutes SaMD.

Lacking other guidance, institutional review boards responsible for research oversight have pointed to FDA standards to determine the risk of research using AI tools. This has led to inconsistent requirements, where researchers and AI companies have the potential to be subject to more scrutiny than EHR vendors even when all three bodies are deploying models with equivalent functionality (and thus similar risk). Clarity is also needed on where responsibility lies for the evaluation, deployment, and monitoring of AI-enabled tools. Health systems are not adequately equipped—either in terms of expertise or resources—to prevent AI-related harms from reaching clinicians and patients.

Why this is important: Without unified guidance, health systems are in the precarious position of having to independently assess whether an AI tool is safe for clinical use. More predictable regulatory pathways would help ensure responsible adoption while reducing legal uncertainty. One area that would especially benefit from a clear regulatory pathway is the use of AI-powered direct-to-patient chatbots and voice AI. These have the potential to expand access to care but may make mistakes. Standardizing how these are evaluated would greatly help health systems interested in adopting these, especially when they lack local AI evaluation expertise. To put this into context, a recent bill proposes to allow states to authorize AI to prescribe medications,⁸ a significant step beyond conversational AI. Before AI is used to practice medicine, there must be clear accountability frameworks for the use of these tools for more basic tasks.

3. Lift restrictions on the use of AI-powered translation tools.

We recommend that restrictions on the use of AI-powered translation tools should be lifted for communicating with individuals who are not fluent in English in a phased manner, commensurate with the improvement of the quality of the AI tools over time. Currently, Title VI of the Civil Rights Act of 1964 and Section 1557 of the Affordable Care Act require the use of interpreters for patients receiving services covered by

the U.S. Department of Health and Human Services.⁹ If a covered entity uses “machine translation,” (i.e., AI), those translations must be reviewed by a qualified human translator to ensure accuracy, “when accuracy is essential,” “when the source documents or materials contain complex, nonliteral or technical language,” or “when the underlying text is critical to the rights, benefits, or meaningful access” of individuals with limited English proficiency (45 CFR § 92.201(c)(3)). We recognize the key role that interpreters play in ensuring accurate in-person communication between clinicians and patients, and that translators play in accurate translation of standardized documents such as consent forms. However, immediate translation services are often not available, particularly in low-resource settings, leading clinicians to defer conversations or use online translation tools that haven’t been properly vetted for use in patient care. AI-powered translation presents a tremendous opportunity to improve access for patients with limited English proficiency.

There has been a substantial shift in how people access healthcare, with much more healthcare taking place via secure text messaging between patients and their care teams than ever before. Secure text messaging is used to address non-urgent concerns, and it is common for multiple short messages to be exchanged to address an issue. For brief messages, AI tools have been shown to be accurate in generating medical translations for a subset of languages, and patients have shown high satisfaction rates with the use of this technology in multiple studies.¹⁰ However, because of regulations restricting the use of AI tools for translation, patients ultimately face a worse experience where messages may be delayed or avoided altogether. Lifting these regulations, if done thoughtfully in a phased approach, will improve language access for patients with limited English proficiency.

We appreciate your consideration of these recommendations. Further, we note that the University of California, UC San Diego Health, and UCSF Health abide by a set of shared principles for the use of AI.^{11,12} We believe that our recommendations are consistent with these principles, including that the use of AI should be appropriate; transparent; accurate, reliable, and safe; fair and non-discriminatory; respectful of privacy and security; reflective of human values; geared towards shared benefit and prosperity; and that we are accountable for its use.

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